

NAVIGATE Presentation Summary

Source: Mueser KT et al. (2015), The NAVIGATE Program for First Episode Psychosis: Rationale, Overview, and Description of Psychosocial Components, *Psychiatric Services*, 66(7), 680-690. Important note: This summary is based only on information stated in the article.

1) Overview / Rationale

NAVIGATE is a coordinated specialty care program for first episode psychosis created as part of the NIMH RAISE Early Treatment Program (ETP). The program was designed specifically for the U.S. mental health system, where services are often fragmented and reimbursement structures differ from the single-payer systems in countries where many early psychosis programs were first developed.

The article explains that people with a first episode of psychosis often face:

- long delays before getting treatment
- confusing and contradictory information
- multiple pathways into care, sometimes through the criminal justice system
- routine services that are not tailored to the developmental and recovery needs of people early in psychosis

NAVIGATE was built to address that gap by providing a team-based, multi-component program that can be implemented in real-world community mental health settings. The overall aim is not just symptom reduction, but helping clients move toward psychological and functional health, regain control over their lives, and pursue personally meaningful goals.

The program is grounded in three major conceptual frameworks named in the paper:

- the recovery model
- the stress-vulnerability model
- psychiatric rehabilitation

These frameworks shape the program's emphasis on:

- personal recovery defined by the client
- education and collaboration
- relapse prevention and illness self-management
- restoring school, work, and social functioning
- hope, strengths, and resiliency rather than a deficit-only approach

2) Presenting Problem It Targets

NAVIGATE targets the broad clinical and psychosocial problems associated with a first episode of psychosis, especially in people early in treatment.

Based on the article, the intervention is meant to address:

- acute and ongoing psychotic symptoms
- confusion and distress after onset of psychosis
- difficulty engaging with treatment
- poor fit between usual services and early psychosis needs
- risk of relapse and rehospitalization
- medication side effects and nonadherence
- family stress and lack of understanding about psychosis
- disruption in school, work, relationships, and community functioning
- substance use, suicidal thinking, trauma reactions, stigma, and demoralization that can accompany psychosis

The paper repeatedly emphasizes that first episode psychosis is not only a symptom problem. It also disrupts identity, autonomy, daily roles, and future plans. NAVIGATE is therefore designed to address both psychiatric stabilization and functional recovery.

3) Target Population

The primary target population is people ages 15 to 35 who have had a first episode of psychosis, especially when schizophrenia-spectrum disorders are most likely to emerge.

The article specifically lists these diagnoses:

- schizophreniform disorder
- schizoaffective disorder
- schizophrenia
- brief psychotic disorder
- psychotic disorder NOS

The program can also include some older individuals, up to age 40, if they have also recently developed a first episode of psychosis.

The article states that NAVIGATE focuses on individuals who:

- are experiencing a first episode of psychosis, regardless of duration of symptoms
- have had no or limited antipsychotic medication exposure, or
- have only recently received treatment and are still early in care

The program is intended for people whose symptoms have:

- remitted or stabilized, and also
- those who still have severe symptoms related to the first episode

However, the article clearly states that NAVIGATE does not include a formal inpatient component. For people needing hospitalization, team members coordinate with inpatient staff and support continuity of care.

4) Major Components of the Intervention

NAVIGATE includes four core interventions.

A. Individualized Medication Treatment

This component uses a shared decision-making approach to medication treatment. The article says the goal is to provide the broadest array of evidence-based options and support decisions based on client preferences and clinical information.

Key features described in the article:

- guideline-based first episode pharmacologic treatment
- attention to low medication doses
- systematic monitoring of signs, symptoms, side effects, and adherence
- continued engagement even if a client wants to stop medication
- support for client-prescriber communication
- attention to medical risks such as metabolic and cardiovascular disease

A major feature is COMPASS, a computerized clinical decision support system that helps gather client-reported information about symptoms, side effects, preferences, and other concerns to guide medication discussions.

B. Family Education Program (FEP)

FEP is designed for relatives or significant others who have regular face-to-face contact with the client. It is meant to build a collaborative alliance between the family and the treatment team and help families become informed, supportive partners in recovery.

The paper describes four stages:

- 1 Engagement, orientation, and assessment
- 2 Stabilization and facilitating recovery
- 3 Consolidating gains
- 4 Promoting prolonged recovery

Core activities include:

- engaging family members early, with client permission
- teaching families about psychosis and treatment
- emphasizing hope, strengths, and resiliency
- reducing family stress
- preventing relapse
- helping family members support client goals and treatment participation
- maintaining monthly contact if no major problems are present
- using family consultation or modified intensive skills training (MIST) when stress or communication problems persist

Sessions may occur in the clinic, at home, or both. The article also notes that FEP can include the client and family together, or the family alone if that is the client's preference.

C. Individual Resiliency Training (IRT)

IRT is the main individual psychotherapy component. It is usually provided weekly or biweekly, in the clinic or community, and focuses on helping clients achieve personal goals while building resiliency, illness self-management, and functioning.

The paper says IRT combines:

- cognitive behavioral therapy methods
- psychoeducation
- motivational enhancement

IRT includes standard modules intended for most clients and individualized modules selected based on need.

Standard IRT modules listed in the paper

- Orientation
- Assessment/Goal-Setting
- Education about Psychosis
- Relapse Prevention Planning
- Processing the Psychotic Episode
- Developing Resiliency
- Building a Bridge to Your Goals

Individualized IRT modules listed in the paper

- Dealing with Negative Feelings
- Coping with Symptoms
- Substance Use
- Having Fun and Developing Good Relationships
- Making Choices about Smoking
- Nutrition and Exercise

The article highlights several distinctive aspects of IRT:

- identifying client strengths early and using them as a foundation
- helping clients process the psychotic episode and its personal impact
- addressing trauma reactions related to psychosis
- challenging self-stigmatizing beliefs through cognitive restructuring
- using positive psychology exercises to build resilience and positive affect

D. Supported Employment and Education (SEE)

SEE addresses a major developmental issue for first episode clients: getting back to school, work, or both. The article says NAVIGATE assumes all clients have these goals, and SEE is available regardless of symptom level.

SEE is based on individual placement and support (IPS) principles, adapted to include educational goals.

The paper describes three broad SEE stages:

- 1 Developing a career and educational profile
- 2 Searching for jobs or educational programs
- 3 Follow-along supports

Key features include:

- services for work, school, or both
- no requirement for prevocational training first
- rapid search after goals are identified
- most services delivered in the community rather than only in the clinic
- practical support with employers, schools, disability services, study skills, and on-the-job supports
- respect for the client's choice about whether to disclose psychiatric information

5) Treatment Team / Roles

The article describes NAVIGATE as a multidisciplinary team, typically with five individuals delivering four core services, while allowing flexibility in staffing and role assignment.

Medication prescriber

Usually a psychiatrist or nurse practitioner. Responsible for:

- individualized medication treatment
- monitoring symptoms, side effects, and signs
- providing guideline-based pharmacologic care

Two IRT clinicians

Usually master's-level clinicians. Responsible for:

- delivering Individual Resiliency Training
- helping clients set personal goals
- teaching illness self-management and recovery skills
- sometimes also providing case management

Supported Employment and Education specialist

Usually bachelor's level. Responsible for:

- helping clients identify and pursue educational goals
- helping clients pursue competitive employment
- providing community-based practical support around school and work

Director

Usually master's level. Responsible for:

- serving as the primary referral liaison
- coordinating and leading the team
- supervising IRT clinicians and the SEE specialist
- usually providing the Family Education Program

Case management

The article notes that case management may naturally occur through IRT clinicians, but a separate sixth team member may also provide it.

Team-based work

Weekly team meetings are used to:

- share updates on progress, stressors, and setbacks
- coordinate care across team members
- support treatment planning and review

The paper frames the small, coordinated team as one of the main ways NAVIGATE helps clients and families navigate both psychosis and the mental health system.

6) Treatment Setting and Implementation Context

NAVIGATE was designed for typical non-academic U.S. mental health care settings, especially community mental health centers that already serve people with serious mental illness.

Important implementation points from the article:

- it is meant for real-world community settings, not only specialty academic programs
- it was developed with the expectation that it should be reimbursable within existing U.S. payment mechanisms
- team members are not expected to be full-time NAVIGATE staff only
- staff may have other responsibilities outside the program
- the model can work as a more dedicated team for larger catchment areas or as a “virtual” team in settings with fewer first episode clients
- clients can also receive other available services outside NAVIGATE, such as peer support or supported housing
- services may be provided in the clinic, home, or broader community depending on the intervention

The article also states that NAVIGATE was evaluated in a cluster randomized controlled trial across 34 sites in 21 states, with participants followed for at least two years.

Although treatment duration is individualized, the paper says most clients are expected to remain in the program for at least one or two years.

7) Measures or Indicators Suggested for Evaluating Treatment Progress or Outcomes

The article is mainly a program description paper, so it does not present a single formal outcome battery in this paper itself. Still, it clearly points to many indicators that can be used to judge progress.

Clinical indicators named or implied in the article

- severity of psychotic symptoms
- relapse occurrence
- hospitalization or rehospitalization
- medication adherence
- medication side effects
- metabolic and cardiovascular risk factors
- substance use
- suicidal thinking or negative feelings when clinically relevant

Functional and recovery indicators named or implied in the article

- progress toward personally meaningful goals
- school enrollment, attendance, or degree progress
- obtaining and maintaining competitive employment
- quality of role functioning
- social and leisure functioning
- quality of relationships and social support
- community integration
- independent living and daily functioning
- sense of well-being, self-esteem, purpose, and environmental mastery

Family-related indicators

- family understanding of psychosis and treatment
- family stress level
- communication and problem-solving within the family
- family support for treatment participation and client goals

Program-process indicators

- engagement in services
- participation in shared decision-making
- use of relapse prevention plans
- continuity of care during transitions such as hospitalization or discharge

Specific examples from the article

- Medication care includes regular monitoring of symptoms, treatment adherence, and side effects.
- FEP emphasizes relapse prevention, stress reduction, and monitoring of family concerns.
- IRT tracks goal-setting and progress and teaches individualized coping and self-management strategies.
- SEE evaluates movement toward work and school goals, including retention and follow-through.

8) Training / Experience Needed to Deliver the Intervention

The article does not give a detailed formal training curriculum for all NAVIGATE staff in this paper, so that should not be overstated. What it does make clear is the following:

Team composition and educational background described in the paper

- medication treatment is provided by a psychiatrist or nurse practitioner
- IRT is usually provided by two master's-level clinicians
- the SEE role is usually bachelor's level
- the director is usually master's level

Skills all team members need

The paper identifies common cross-team competencies, including:

- shared decision-making skills
- strengths and resiliency focus
- motivational enhancement skills
- psychoeducational skills
- collaboration with family and other natural supports

Program materials and standardization

The article states that NAVIGATE is standardized in six manuals, including:

- a team members' guide
- a director manual
- a manual for medication treatment
- a manual for FEP
- a manual for IRT

- a manual for SEE

Training implications that are explicitly supported by the paper

To deliver NAVIGATE as described, staff would need familiarity with:

- first episode psychosis care
- recovery-oriented and shared decision-making practice
- CBT- and motivational interviewing-informed strategies used in IRT
- family psychoeducation and consultation approaches
- supported employment / supported education principles
- guideline-based medication treatment for first episode psychosis
- teamwork and coordinated care planning

The paper also explicitly notes that the SEE manual included both educational/employment guidance and the training of SEE specialists.

9) Strengths / Innovations

The article presents several major strengths and innovations of NAVIGATE.

Specifically tailored for first episode psychosis

The program is not generic community care. It is designed around the developmental and clinical needs of people early in psychosis.

Designed for the U.S. system

This is one of the paper's biggest selling points. NAVIGATE was built to work inside the fragmented, multipayer U.S. healthcare context rather than assuming a unified national system.

Comprehensive but practical

It integrates:

- medication treatment
- psychotherapy / resiliency training
- family work
- work and school support

That makes it broad enough to address real recovery while still being described as feasible for usual care settings.

Strong recovery orientation

The article repeatedly emphasizes:

- hope
- person-centered care
- self-determination
- personally meaningful goals
- moving beyond symptom control alone

Shared decision-making as a core value

The model treats the client as an active partner rather than a passive recipient of care.

Strengths and resiliency emphasis

This is a notable innovation compared with deficit-focused treatment models. The program deliberately identifies strengths and uses them in care planning.

Attention to trauma and stigma

IRT explicitly includes processing the psychotic episode, addressing trauma reactions, and challenging self-stigmatizing beliefs.

Inclusion of positive psychology in IRT

The use of positive psychology exercises to build resilience and positive affect is presented as a distinctive feature.

Direct focus on work and school

SEE addresses educational and employment goals early rather than treating them as later-stage issues.

Measurement-based medication support through COMPASS

The computerized decision support system is a practical innovation intended to improve evidence-based medication care and client-prescriber communication.

10) Limitations or Practical Considerations

The article is generally optimistic, but it also points to real-world limitations and implementation issues.

No formal inpatient component

NAVIGATE does not include inpatient treatment. It depends on coordination with hospitals when clients need that level of care.

Staffing realities in community settings

Most sites cannot support a fully dedicated early psychosis team, so staff may split time across programs. That improves feasibility, but may also create workload and coordination challenges.

Small target population in some catchment areas

Because the number of first episode clients can be limited, some sites may need a “virtual” team rather than a fully dedicated one.

Education outcomes were not yet firmly established in the larger literature

The article notes that while supported employment had encouraging evidence, educational gains had not yet been clearly demonstrated, which is part of why SEE required specialized educational guidance.

Family involvement depends on client permission and available supports

FEP is offered with the client’s permission and assumes the presence of involved relatives or significant others. Not every client will have that support available.

Client choice can make service pathways non-linear

Clients can choose which interventions they want, and when to start, stop, or resume them. That is recovery-oriented, but it also means treatment may not proceed in a uniform way.

This paper is descriptive, not the outcomes paper

The article describes rationale and intervention structure. It references an ongoing randomized trial, but this paper itself is not the trial outcome report.

11) Extra Slide-Worthy Talking Points

- NAVIGATE is about more than symptom reduction; it is about helping people reclaim direction, identity, and functioning after a first episode of psychosis.
- The name “NAVIGATE” itself reflects the idea of guiding people through both mental illness and a confusing healthcare system.
- The program was intentionally built for community mental health settings, not just elite specialty centers.
- Clients are not forced into a one-size-fits-all track; they can choose among services and decide when to engage in each one.
- Family support is treated as a clinical asset, not an afterthought.
- Education and employment are framed as central recovery goals early on, not something to postpone until “after stabilization.”
- IRT is notable for combining CBT, motivational enhancement, psychoeducation, trauma processing, anti-stigma work, and positive psychology.
- The model balances recovery values with structure: manuals, team meetings, role clarity, and measurement-based medication support.
- NAVIGATE’s flexibility in staffing was part of its design, which matters for dissemination in ordinary U.S. systems.

Verbatim Quotes from the Article for Speaker Notes

“NAVIGATE is a team-based, multi-component treatment program designed to be implemented in routine mental health treatment settings and aimed at guiding people with a first episode of psychosis (and their families) towards psychological and functional health.” (Abstract, p. 680)

“The core services provided in the NAVIGATE program include the Family Education Program, Individual Resiliency Training, Supported Employment and Education, and Individualized Medication Treatment.” (Abstract, p. 680)

“NAVIGATE embraces a shared decision-making approach with a focus on strengths and resiliency, and collaboration with clients and family members in treatment planning and reviews.” (Abstract, p. 680)

“The NAVIGATE program is aimed at helping clients (and their family members) negotiate their way through the haze of mental illness and the maze of the mental healthcare system, through close collaboration with a small and committed treatment team, towards regaining control over their lives and achieving their personal goals.” (p. 683-684)

“NAVIGATE is a coordinated specialty care program aimed at helping people with a first episode of psychosis meet the broad range of their psychiatric and psychosocial needs.” (p. 684)

“The NAVIGATE program was designed to be implemented in typical non-academic U.S. mental health care settings serving the broad population of persons with a serious mental illness (e.g., community mental health centers).” (p. 684)

“All of the services provided in NAVIGATE are individualized, based on the client’s goals and needs.” (p. 685)

“The philosophy, goals, and services of NAVIGATE are guided by three broad conceptual frameworks in the mental health field, including the recovery model, the stress-vulnerability model, and the general field of psychiatric rehabilitation...” (p. 685)

“Thus, the goal of the NAVIGATE program is recovery, defined by each individual in their own terms...” (p. 686)

“Clients are free to choose which of the NAVIGATE interventions they wish to receive (Individualized Medication Treatment, FEP, IRT, and SEE), and when to start, stop, and resume each one.” (p. 688)

“A unique aspect of IRT is the incorporation of exercises from positive psychology into the curriculum...” (p. 690)

“In this model, small teams of providers facilitate recovery by building skills for individual resiliency, reinforcing natural supports, providing practical help in achieving work and educational goals, and providing tailored pharmacological treatment specifically designed for people with a first episode of psychosis.” (p. 692)

Concise Bullet Points That Can Go Directly on Slides

Slide-ready summary

- NAVIGATE = coordinated specialty care for first episode psychosis

- Developed through NIMH RAISE for the U.S. mental health system
- Built for community mental health settings
- Focuses on recovery, shared decision-making, strengths, and resiliency
- Targets both symptoms and functional recovery

Core components

- Individualized Medication Treatment
- Family Education Program (FEP)
- Individual Resiliency Training (IRT)
- Supported Employment and Education (SEE)

Who it serves

- Primarily ages 15-35 with a first episode of psychosis
- Can include some clients up to age 40
- Designed for people early in treatment, including those with limited antipsychotic exposure

Team model

- Small multidisciplinary team
- Prescriber
- 2 IRT clinicians
- SEE specialist
- Director / team leader
- Weekly team meetings for coordination

What makes it different

- Recovery is defined by the client's own goals
- Strong emphasis on family involvement and natural supports
- Direct attention to school and work early in recovery
- Uses COMPASS for measurement-based medication support
- IRT includes trauma processing, anti-stigma work, and positive psychology

Key outcomes to discuss

- Symptom stabilization
- Relapse and hospitalization prevention
- Medication adherence and side-effect monitoring
- Return to school or work
- Progress toward personal goals
- Improved family support and reduced stress

Practical considerations

- No formal inpatient component
- Flexible staffing model for real-world settings
- Clients choose which services to use and when
- Descriptive paper; outcome trial discussed separately

Quick Speaker Notes / Plain-English Bottom Line

If you need a simple way to explain NAVIGATE out loud: it is a whole-person early psychosis program built to help someone after a first psychotic episode not only get clinically stable, but also rebuild life direction. Instead of treating psychosis as only a medication problem, NAVIGATE combines medication support, family work, psychotherapy focused on resiliency, and concrete help returning to school or work. The model is especially important because it was designed to be realistic for ordinary U.S. community mental health settings.